Adult COPD / Asthma

AR-04 | Airway & Respiratory

Quick flow

01 Respiratory Distress [EMT / AEMT / Paramedic]

Assess work of breathing, speech, air movement, SpO_2, lung sounds, mental status, and fatigue.

02 Wheeze, Stridor, or Another Cause? [EMT / AEMT / Paramedic]

Treat anaphylaxis immediately when suspected; assess for CHF, ACS, PE, pneumonia, pneumothorax, obstruction, or foreign body.

03 Oxygen + Bronchodilator [EMT / AEMT / Paramedic]

Position upright • target oxygen appropriately • albuterol ± ipratropium.

04 NIPPV Appropriate? [AEMT / Paramedic]

AEMT/Paramedic: use early if cooperative and stable; do not use with emesis, inability to protect airway, or shock.

05 Severe / Not Improving? [AEMT / Paramedic]

Epinephrine only under the indication- and provider-specific orders below • BVM support • request Paramedic escalation.

06 Paramedic - Severe Bronchospasm [Paramedic]

Methylprednisolone • magnesium when indicated • advanced-airway preparation; follow doses and safeguards below.

07 Reassess + Transport [EMT / AEMT / Paramedic]

Continuous SpO_2; $EtCO_2$ when indicated; reassess after each treatment and notify early.

Overview

- Differentiate lower-airway bronchospasm from stridor and other causes of respiratory distress while supporting oxygenation and ventilation.
- Escalate early for silent chest, fatigue, altered mental status, poor air movement, or impending respiratory failure.

Provider-Level Actions

Use the least invasive effective airway intervention and escalate only when oxygenation or ventilation remains inadequate.

EMT

- Position for comfort, provide oxygen, assist ventilation when tiring, and administer authorized albuterol.
- Use epinephrine immediately for anaphylaxis within EMT scope and request ALS for severe distress or poor response.
- Monitor work of breathing, speech, mental status, air movement, SpO_2, and clinical response after every intervention.

AEMT

- Perform all EMT care plus NIPPV for an appropriate cooperative, hemodynamically stable patient; obtain vascular access and give repeated albuterol/ipratropium within scope.
- Use authorized IM epinephrine for anaphylaxis or life-threatening asthma. Request Paramedic care for nebulized epinephrine for stridor/upper-airway edema; the nebulized route is Paramedic-only.

Paramedic

- Perform all prior care plus nebulized epinephrine for stridor/upper-airway edema, cardiac and $EtCO_2$ monitoring when indicated, steroid and magnesium therapy, advanced-airway management, and treatment of competing diagnoses.
- Prepare BVM with PEEP and follow the airway pathway promptly for silent chest, exhaustion, declining mental status, or ineffective ventilation.

Indications

- Adult with respiratory distress from suspected asthma, COPD, reactive airway disease, bronchospasm, or stridor.

Treatment steps

- Target SpO_2 93-95% for asthma or undifferentiated hypoxemia. For known COPD with hypercapnia risk, usual lower saturation, or an individualized oxygen plan, target 88-92%; do not withhold oxygen from a critically hypoxemic patient.
- Give albuterol promptly; add ipratropium to the initial treatment for moderate or severe bronchospasm. Repeat bronchodilator treatment according to clinical response.
- Use NIPPV early for a cooperative patient with significant distress, fatigue, or hypercapnic COPD physiology. Stop NIPPV and move to BVM/airway-resuscitation management for hypotension, worsening mental status, emesis, intolerance, or deteriorating ventilation.
- For severe asthma or COPD exacerbation, give methylprednisolone early; it is adjunctive and must not delay bronchodilator treatment.
- Use magnesium sulfate for severe asthma or bronchospasm not improving after initial bronchodilator therapy; monitor for hypotension.
- Use IM epinephrine for anaphylaxis, severe upper-airway edema, or life-threatening asthma with poor air movement or impending respiratory failure. Paramedic only: use nebulized epinephrine for stridor or upper-airway edema, not routine lower-airway COPD/asthma treatment.
- Reassess respiratory effort, ability to speak, air movement, SpO_2, $EtCO_2$ when used, heart rate, and blood pressure after each intervention. Transport all severe, recurrent, or incompletely responsive patients and notify early for NIPPV, epinephrine, magnesium, or impending airway failure.

Medications

Albuterol

2.5-5 mg nebulized; repeat as needed ×3 and continue when clinically indicated

Ipratropium

0.5 mg nebulized with the initial albuterol treatment for moderate or severe bronchospasm

Epinephrine 1 mg/mL (1:1,000)

0.3-0.5 mg IM for anaphylaxis, severe upper-airway edema, or life-threatening asthma

Nebulized epinephrine - Paramedic only

1 mg of 1 mg/mL (1:1,000) in 2 mL normal saline; may repeat once for stridor or upper-airway edema

Methylprednisolone

125 mg IV/IO/IM

Magnesium sulfate

1-2 g IV over 15-30 minutes for severe asthma/bronchospasm not improving after initial bronchodilators

Warnings / precautions

- A silent chest with respiratory distress is a pre-arrest finding.

- Do not persist with NIPPV in a patient with hypotension, worsening mental status, emesis, inability to protect the airway, intolerance, or deteriorating ventilation.
- Do not delay assisted ventilation and airway preparation for repeated nebulizer treatments in a tiring patient.
- Medication administration must follow current Claiborne authorization and standing orders.

Clinical pearls

- A patient who can no longer speak, is tiring, or has decreasing air movement may be worsening even when wheezing is quieter.
- Do not treat every wheeze as asthma/COPD; reassess for anaphylaxis, CHF, ACS, PE, pneumothorax, pneumonia, upper-airway obstruction, and foreign body.

Related pathways / contacts

AR-01 Adult Airway - Open for progressive respiratory failure or advanced-airway support.

AR-03 Drug-Assisted Airway - Open only when DAI is indicated and authorized for impending or actual airway failure.

AM-01 Allergic Reaction / Anaphylaxis - Open for suspected anaphylaxis or airway edema.

AM-05 Hypotension / Shock - Open for persistent hypotension or shock.

AM-07 Crashing Medical Patient - Open for immediate physiologic resuscitation in a critically unstable patient.

Source and references

Claiborne County EMS approved protocol manual. | Claiborne County EMS formulary, linked protocol pathways, and agency release guidance.

Revision: October 17, 2026 | Last verified: 2026-10-31